Patient Record — ID 1764

Age	28	**Sex**	F
Race/Ethnicity	Non-Hispanic White	**Insurance**	Blue Cross Blue Shield PPO
Total Encounters	8	**Generation Seed**	3725

Primary Diagnoses

• Breech presentation at term
• Asymptomatic Actinomyces colonization of genital tract
• Malignant melanoma
• Anencephaly
• High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy
• Group B Streptococcus carrier state complicating pregnancy
• Pregnancy (20 weeks gestation)
• False labor (Braxton Hicks contractions)

Comorbidities

• Term pregnancy
• Pregnancy (20 weeks gestation)
• Pregnancy, 38 weeks gestation (third trimester)
• Nulliparity

Patient Profile

Sex	F
Age at First Encounter	28
Race/Ethnicity	Non-Hispanic White
Insurance	Blue Cross Blue Shield PPO
Occupation	Elementary school teacher
Smoking Status	Never smoker
Alcohol Use	None

Allergies:

• No known drug allergies

Chronic Conditions:

• Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
• History of malignant melanoma (back, status post wide local excision)
• History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Home Medications:

• Carbamazepine - 200 mg PO twice daily
• Folic acid - 4 mg PO daily

Surgical History:

• Wide local excision with 1cm margins, right upper back (melanoma)
• Cervical colposcopy (unsatisfactory visualization)
• Sentinel lymph node biopsy, negative for malignancy

Family History:

• Maternal aunt: Malignant melanoma (age 45)
• Paternal grandmother: Epilepsy
• Mother: Hypothyroidism
• Father: Type 2 diabetes mellitus

Encounters (8)

Encounter 1 — 2020-01-15 — Dermatology

Encounter ID	7343	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Dermatology	**Gen. Method**	template

Chief Complaint:

Concerns about a changing mole on her back that her husband recently noticed has become darker and slightly larger over the past two months.

Full Note:

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ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Dermatology
Visit Type: outpatient

CHIEF COMPLAINT:
Concerns about a changing mole on her back that her husband recently noticed has become darker and
slightly larger over the past two months.

HISTORY OF PRESENT ILLNESS:
She reports that the mole has become darker and slightly larger over the past two months. She
denies any pain, itching, or bleeding from the site.

PAST MEDICAL HISTORY:
Active Problem List:
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies
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FAMILY HISTORY:
She has no family history of skin cancer.

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On examination, the mole measures 8 mm in diameter, has irregular borders, and exhibits variegated colors including brown, black, and a hint of blue. There is no evidence of ulceration or bleeding. The remainder of her skin examination is normal.

ASSESSMENT AND PLAN:
Given the changes in the mole during pregnancy,

Encounter 2 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	7345	**Type**	outpatient
Attending	Dr. Maria Rodriguez	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Routine gynecologic examination with abnormal cervical cancer screening results requiring colposcopic evaluation.

Full Note:

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ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. Maria Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine gynecologic examination with abnormal cervical cancer screening results requiring
colposcopic evaluation.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.
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Encounter 3 — 2021-07-15 — Obstetrics & Gynecology

Encounter ID	7342	**Type**	outpatient
Attending	Dr. Jennifer Park	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine annual well-woman examination with no active complaints.

Full Note:

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ENCOUNTER NOTE
Date: 2021-07-15
Provider: Dr. Jennifer Park, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine annual well-woman examination with no active complaints.

HISTORY OF PRESENT ILLNESS:
The patient presents for routine annual well-woman examination with a history of generalized
seizure disorder, malignant melanoma status post wide local excision, and high-grade squamous
intraepithelial lesion of the cervix following prior dermatologic evaluation for a changing back
mole and gynecologic assessment for abnormal cervical cancer screening. She has no complaints.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

She has never been pregnant and has regular menstrual cycles. Her last Pap smear was two years ago
and was normal.

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
She is in a monogamous relationship

VITAL SIGNS:
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On examination, her vital signs are within normal limits

PHYSICAL EXAMINATION:
Her pelvic exam is unremarkable with no cervical motion tenderness or adnexal masses

PATHOLOGY:
A Pap smear is performed, and the results show the presence of Actinomyces-like organisms.

ASSESSMENT AND PLAN:
The patient remains asymptomatic.

Encounter 4 — 2022-05-15 — Obstetrics & Gynecology

Encounter ID	7347	**Type**	outpatient
Attending	Dr. Emily Johnson	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

First prenatal visit at approximately 10 weeks gestation with mild nausea and fatigue.

Full Note:

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ENCOUNTER NOTE
Date: 2022-05-15
Provider: Dr. Emily Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
First prenatal visit at approximately 10 weeks gestation with mild nausea and fatigue.

HISTORY OF PRESENT ILLNESS:
This 30-year-old patient with epilepsy, history of malignant melanoma, and high-grade cervical
dysplasia presents for her first prenatal visit following prior gynecologic care. She reports
feeling well with mild nausea and fatigue.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her immunizations are up to date.

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
She has no known drug allergies

FAMILY HISTORY:
Her family history is non-contributory.

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITAL SIGNS:
On physical examination, her blood pressure is 120/78 mmHg, heart rate is 72 beats per minute, and
she is afebrile.

PHYSICAL EXAMINATION:
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Her abdomen is soft, non-tender, with no palpable masses. Pelvic examination is normal for gestational age.

LABORATORY DATA:
Laboratory results are as follows:

- Blood type: A negative
- Antibody screen: Negative
- Hemoglobin: 12.5 g/dL
- White blood cell count: 7,000/uL
- Platelets: 250,000/uL

ASSESSMENT AND PLAN:
Given the patient's blood type and antibody screen,

Encounter 5 — 2022-08-15 — Obstetrics & Gynecology

Encounter ID	7344	**Type**	outpatient
Attending	Dr. Emily Johnson	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit at 20 weeks gestation for second-trimester screening.

Full Note:

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ENCOUNTER NOTE
Date: 2022-08-15
Provider: Dr. Emily Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit at 20 weeks gestation for second-trimester screening.

HISTORY OF PRESENT ILLNESS:
She has melanoma, HSIL, epilepsy. She returns for 20-week visit reporting feeling well with
uneventful pregnancy thus far.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her medical history is significant for epilepsy, for which she takes carbamazepine.

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
She has been taking prenatal vitamins since she found out she was pregnant at 8 weeks gestation.

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
She does not smoke or drink alcohol.
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VITAL SIGNS:
On examination, her blood pressure is 120/78 mmHg, heart rate is 72 beats per minute,

PHYSICAL EXAMINATION:
Fetal heart tones are normal.

IMAGING:
A routine fetal ultrasound is performed, which reveals an absent cerebrum, a calvarial defect, and an abnormal cerebellum. There is also evidence of polyhydramnios.

Encounter 6 — 2022-12-15 — Emergency Medicine

Encounter ID	7348	**Type**	ed
Attending	Dr. David Kim	**Order**	5
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

Mild, irregular contractions every 10 to 15 minutes for the past 3 hours without vaginal bleeding or fluid leakage.

Full Note:

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ENCOUNTER NOTE
Date: 2022-12-15
Provider: Dr. David Kim, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
Mild, irregular contractions every 10 to 15 minutes for the past 3 hours without vaginal bleeding
or fluid leakage.

HISTORY OF PRESENT ILLNESS:
This 30-year-old patient with new-onset epilepsy and history of malignant melanoma and cervical
HSIL presents at 36 weeks gestation. She describes the contractions as mild and irregular,
occurring approximately every 10 to 15 minutes, without any increase in intensity. She denies any
vaginal bleeding, leakage of fluid, or decrease in fetal movement.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her pregnancy has been uncomplicated, and prenatal care has been adequate.

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
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Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITAL SIGNS:
On physical examination, her vital signs are stable,

PHYSICAL EXAMINATION:
The fetal heart rate tracing is reassuring. A speculum examination reveals no evidence of ruptured membranes, and the cervix is closed, long, and posterior. No cervical change is noted compared to her last prenatal visit a week ago.

LABORATORY DATA:
Laboratory Values:
- Hemoglobin: 12.2 g/dL
- White blood cell count: 9,000/mm^3
- Platelets: 250,000/mm^3
- Urinalysis: No proteinuria or glucosuria

Encounter 7 — 2023-02-15 — Obstetrics & Gynecology

Encounter ID	7341	**Type**	outpatient
Attending	Dr. Lisa Thompson	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit at term with discussion of delivery plans given suspected fetal malpresentation.

Full Note:

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ENCOUNTER NOTE
Date: 2023-02-15
Provider: Dr. Lisa Thompson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit at term with discussion of delivery plans given suspected fetal
malpresentation.

HISTORY OF PRESENT ILLNESS:
This 31-year-old with malignant melanoma, cervical HSIL, and epilepsy returns for prenatal care at
term. She reports feeling well with good fetal movement. Her pregnancy has been uncomplicated, and
she has received regular prenatal care. The patient expresses a desire for a vaginal delivery if
possible.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- False labor (Braxton Hicks contractions) (diagnosed 2022-12-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.
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VITAL SIGNS:
On examination, her blood pressure is 120/78 mmHg, heart rate is 72 bpm, and fetal heart rate is 140 bpm.

PHYSICAL EXAMINATION:
The obstetric examination reveals a fundal height consistent with gestational age, but the fetal position is noted to be breech on Leopold maneuvers.

IMAGING:
An ultrasound confirms a frank breech presentation with no other abnormalities. Amniotic fluid index is normal, and there are no signs of placental insufficiency.

Encounter 8 — 2023-03-15 — Obstetrics & Gynecology

Encounter ID	7346	**Type**	inpatient
Attending	Dr. Robert Garcia	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Regular contractions every 5 minutes for 2 hours with spontaneous rupture of membranes 30 minutes prior.

Full Note:

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ENCOUNTER NOTE
Date: 2023-03-15
Provider: Dr. Robert Garcia, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Regular contractions every 5 minutes for 2 hours with spontaneous rupture of membranes 30 minutes
prior.

HISTORY OF PRESENT ILLNESS:
The 31-year-old with melanoma, HSIL, and epilepsy since 12 weeks gestation returns following
breech presentation. She reports her water broke 30 minutes ago. Her prenatal course was
complicated by asymptomatic bacteriuria at 16 weeks caused by GBS and treated with amoxicillin.

PAST MEDICAL HISTORY:
Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed
2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- False labor (Braxton Hicks contractions) (diagnosed 2022-12-15)
- Breech presentation at term (diagnosed 2023-02-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PAST SURGICAL HISTORY:
- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS:
Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.
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VITAL SIGNS:
Her temperature is 37.2°C (99.0°F), blood pressure is 120/78 mmHg, pulse is 88/min, and respiratory rate is 16/min.

PHYSICAL EXAMINATION:
Physical examination reveals a gravid abdomen with a fundal height appropriate for gestational age. Fetal heart tones are regular at 140 beats/min. Pelvic examination shows that the cervix is dilated to 4 cm, effaced 80%, and the amniotic membranes are ruptured.

ASSESSMENT AND PLAN:
Given her medical history,